

## **1. NAME OF THE MEDICINAL PRODUCT**

Femoston® Conti 0.5mg/2.5mg film-coated tablets

## **2. QUALITATIVE AND QUANTITATIVE COMPOSITION**

28 tablets, each containing 0.5 mg 17β-estradiol (as hemihydrate) and 2.5 mg dydrogesterone.

Excipient(s): Lactose monohydrate.

For a full list of excipients, see section 6.1.

## **3. PHARMACEUTICAL FORM**

Film-coated tablet.

A round, biconvex, marked 379 on one side.

Yellow 0.5/2.5 mg tablets.

## **4. CLINICAL PARTICULARS**

### **4.1 Therapeutic Indications**

Hormone replacement therapy (HRT) for oestrogen deficiency symptoms in postmenopausal women at least 12 months since last menses.

#### Elderly population

The experience in treating women older than 65 years is limited.

### **4.2 Posology and Method of Administration**

For oral use.

Femoston® Conti 0.5mg/2.5mg is a continuous combined HRT for oral use.

The oestrogen and the progestogen are given every day without interruption.

The dosage is one tablet per day for a 28-day cycle.

Femoston® Conti 0.5mg/2.5mg should be taken continuously without a break between packs.

For initiation and continuation of treatment of postmenopausal symptoms, the lowest effective dose for the shortest duration (see also section 4.4) should be used.

Continuous combined treatment may be started with Femoston® Conti 0.5 mg/2.5mg depending on the time since menopause and severity of symptoms.

Women experiencing a natural menopause should commence treatment with Femoston® Conti 0.5 mg/2.5mg not earlier than at least 12 months after their last natural menstrual bleed. For surgically induced menopause, treatment may start immediately.

Depending on the clinical response, the dosage can subsequently be adjusted.

Patients changing from a continuous sequential or cyclical preparation should complete the 28-day cycle and then change to Femoston® Conti 0.5mg/2.5mg.

Patients changing from another continuous combined preparation may start therapy at any time.

If a dose has been forgotten, it should be taken as soon as possible. If more than 12 hours have elapsed, treatment should be continued with the next tablet without taking the forgotten tablet. The likelihood of breakthrough bleeding or spotting may be increased.

Femoston® Conti 0.5mg/2.5mg can be taken irrespectively of food intake.

#### Paediatric population:

There is no relevant indication for the use of Femoston® Conti 0.5mg/2.5mg in the paediatric population.

### **4.3 Contraindications**

- Known past or suspected breast cancer
- Known or suspected estrogen-dependent malignant tumours (e.g., endometrial cancer)
- Known or suspected progestogen-dependent neoplasms
- Meningioma or history of meningioma
- Undiagnosed genital bleeding
- Untreated endometrial hyperplasia
- Previous or current venous thromboembolism (deep venous thrombosis, pulmonary embolism)
- Known thrombophilic disorders (e.g., protein C, protein S or antithrombin deficiency, see section 4.4)
- Active or recent arterial thromboembolic disease (e.g., angina, myocardial infarction)
- Acute liver disease, or a history of liver disease, as long as the liver function tests have failed to return to normal
- Porphyria
- Known hypersensitivity to the active substances or to any of the excipients

### **4.4 Special Warnings and Precautions for Use**

For the treatment of postmenopausal symptoms, HRT should only be initiated for symptoms that adversely affect quality of life. In all cases, a careful appraisal of the risks and benefits should be undertaken at least annually and HRT should only be continued as long as the benefit outweighs the risk.

Evidence regarding the risks associated with HRT in the treatment of premature menopause is limited. Due to the low level of absolute risk in younger women, however, the balance of benefits and risks for these women may be more favourable than in older women.

#### Medical examination/follow-up

Before initiating or re-instituting HRT, a complete personal and family medical history should be taken. Physical (including pelvic and breast) examination should be guided by this and by the contraindications and warnings for use. During treatment, periodic check-ups are recommended of a frequency and nature adapted to the individual woman. Women should be advised what changes in their breasts should be reported to their doctor or nurse (see “Breast cancer” below). Investigations, including appropriate imaging tools, e.g., mammography should be carried out in accordance with currently accepted screening practices, modified to the clinical needs of the individual.

#### Conditions which need supervision

If any of the following conditions are present, have occurred previously, and/or have been aggravated during pregnancy or previous hormone treatment, the patient should be closely supervised. It should be taken into account that these conditions may recur or be aggravated during treatment with Femoston® Conti 0.5mg/2.5mg, in particular:

- Leiomyoma (uterine fibroids) or endometriosis
- Risk factors for thromboembolic disorders (see below)
- Risk factors for oestrogen dependent tumours, e.g., 1st degree heredity for breast cancer
- Hypertension
- Liver disorders (e.g., liver adenoma)
- Diabetes mellitus with or without vascular involvement
- Cholelithiasis
- Migraine or (severe) headache
- Systemic lupus erythematosus
- A history of endometrial hyperplasia (see below)
- Epilepsy
- Asthma
- Otosclerosis

### Meningioma

The occurrence of meningiomas (single and multiple) has been reported in association with use of Estradiol/Dydrogesterone. Patients should be monitored for signs and symptoms of meningiomas in accordance with clinical practice. If a patient is diagnosed with meningioma, any Estradiol/Dydrogesterone containing treatment must be stopped (see section 4.3). Tumour shrinkage has been observed after treatment discontinuation.

### Reasons for immediate withdrawal of therapy:

Therapy should be discontinued in case a contraindication is discovered and in the following situations:

- Jaundice or deterioration in liver function
- Significant increase in blood pressure
- New onset of migraine-type headache
- Pregnancy

### Endometrial hyperplasia and carcinoma

- In women with an intact uterus the risk of endometrial hyperplasia and carcinoma is increased when estrogens are administered alone for prolonged periods. The reported increase in endometrial cancer risk among oestrogen-only users varies from 2- to 12- fold greater compared with non-users, depending on the duration of treatment and oestrogen dose (see Section 4.8). After stopping treatment risk may remain elevated for at least 10 years.
- The addition of a progestogen cyclically for at least 12 days per month /28-day cycle or continuous combined oestrogen-progestogen therapy in non-hysterectomised women can prevent the excess risk associated with oestrogen-only HRT.
- Breakthrough bleeding and spotting may occur during the first months of treatment. If breakthrough bleeding or spotting appears after some time on therapy, or continues after treatment has been discontinued, the reason should be investigated, which may include endometrial biopsy to exclude endometrial malignancy.

### Breast cancer

The overall evidence shows an increased risk of breast cancer in women taking combined oestrogen-progestogen or oestrogen-only HRT, that is dependent on the duration of taking HRT.

### Combined oestrogen-progestogen therapy

- The randomised placebo-controlled trial, the Women's Health Initiative study (WHI), and a meta-analysis of prospective epidemiological studies are consistent in finding an increased risk of breast cancer in women taking combined oestrogen-progestogen for HRT that becomes apparent after about 3(1-4) years (see Section 4.8).

### Oestrogen-only therapy

- The WHI trial found no increase in the risk of breast cancer in hysterectomised women using oestrogen-only HRT. Observational studies have mostly reported a small increase in risk of having breast cancer diagnosed that is substantially lower than that found in users of oestrogen-progestogen combinations (see Section 4.8).

Results from a large meta-analysis showed that after stopping treatment the excessive risk will decrease with time and the time needed to return to baseline depends on duration of prior HRT use. When HRT was taken for more than 5 years, the risk may persist for 10 years or more. HRT, especially oestrogen-progestogen combined treatment, increases the density of mammographic images which may adversely affect the radiological detection of breast cancer.

### **Ovarian cancer**

Ovarian cancer is much rarer than breast cancer. Epidemiological evidence from a large meta-analysis suggests a slightly increased risk in women taking oestrogen-only or combined oestrogen-progestogen HRT, which becomes apparent within 5 years of use and diminishes over time after Stopping. Some other studies, including the WHI trial suggest that use of combined HRTs may be associated with a similar, or slightly smaller, risk (see Section 4.8).

### **Venous thromboembolism**

- HRT is associated with a 1.3-3 fold risk of developing venous thromboembolism (VTE), i.e. deep vein thrombosis or pulmonary embolism. The occurrence of such an event is more likely in the first year of HRT than later (see Section 4.8).
- Patients with known thrombophilic states have an increased risk of VTE and HRT may add to this risk. HRT is therefore contraindicated in these patients (see section 4.3).
- Generally recognised risk factors for VTE include: use of oestrogens, older age, major surgery, prolonged immobilisation, obesity (BMI > 30 kg/m<sup>2</sup>), pregnancy/postpartum period, systemic lupus erythematosus (SLE), and cancer. There is no consensus about the possible role of varicose veins in VTE.
- As in all postoperative patients, prophylactic measures need be considered to prevent VTE following surgery. If prolonged immobilisation is to follow elective surgery temporarily stopping HRT 4 to 6 weeks earlier is recommended. Treatment should not be restarted until the woman is completely mobilised.
- In women with no personal history of VTE but with a first degree relative with a history of thrombosis at young age, screening may be offered after careful counselling regarding its limitations (only a proportion of thrombophilic defects are identified by screening).
- If a thrombophilic defect is identified which segregates with thrombosis in family members or if the defect is 'severe' (e.g. antithrombin, protein S, or protein C deficiencies or a combination of defects) HRT is contraindicated.
- Women already on chronic anticoagulant treatment require careful consideration of the benefit-risk of use of HRT.
- If VTE develops after initiating therapy, the drug should be discontinued. Patients should be told to contact their doctors immediately when they are aware of a potential thromboembolic-symptom (e.g. painful swelling of a leg, sudden pain in the chest dyspnoea).

### **Coronary artery disease (CAD)**

There is no evidence from randomised controlled trials of protection against myocardial infarction in women with or without existing CAD who received combined oestrogen-progestogen or oestrogen-only HRT.

#### **Combined oestrogen-progestogen therapy**

The relative risk of CAD during use of combined oestrogen-progestogen HRT is slightly increased. As the baseline absolute risk of CAD is strongly dependent on age, the number of extra cases of CAD due to oestrogen-progestogen use is very low in healthy women close to menopause, but will rise with more advanced age.

#### **Oestrogen-only**

Randomised controlled data found no increased risk of CAD in hysterectomised women using oestrogen-only therapy.

### **Ischaemic Stroke**

Combined oestrogen-progestogen and oestrogen-only therapy are associated with an up to 1.5-fold increase in risk of ischaemic stroke. The relative risk does not change with age or time since menopause. However, as the baseline risk of stroke is strongly age-dependent, the overall risk of stroke in women who use HRT will increase with age (see Section 4.8).

### **Other conditions**

- Oestrogens may cause fluid retention and therefore patients with cardiac or renal dysfunction should be carefully observed.
- Women with pre-existing hypertriglyceridaemia should be followed closely during oestrogen replacement or hormone replacement therapy, since rare cases of large increases of plasma triglycerides leading to pancreatitis have been reported with oestrogen therapy in this condition.
- Exogenous estrogens may induce or exacerbate symptoms of hereditary and acquired angioedema.
- Oestrogens increase thyroid binding globulin (TBG), leading to increased circulating total thyroid hormone, as measured by protein-bound iodine (PBI), T4 levels (by column or by radio-immunoassay) or T3 levels (by radio-immunoassay). T3 resin uptake is decreased, reflecting the elevated TBG. Free T4 and free T3 concentrations are unaltered. Other binding proteins may be elevated in serum, i.e., corticoid binding globulin (CBG), sexhormone-binding globulin (SHBG) leading to increased circulating corticosteroids and sex steroids, respectively. Free or biological active hormone concentrations are unchanged. Other plasma proteins may be increased (angiotensinogen/renin substrate, alpha-1antitrypsin, ceruloplasmin).
- HRT use does not improve cognitive function. There is some evidence of increased risk of probable dementia in women who start using continuous combined or estrogen-only HRT after the age of 65.
- Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.

This oestrogen-progestogen combination treatment is not contraceptive.

### **ALT elevations**

During clinical trials with patients treated for hepatitis C virus (HCV) infections with the combination regimen ombitasvir/paritaprevir/ritonavir and dasabuvir with and without ribavirin, ALT elevations greater than 5 times the upper limit of normal (ULN) were significantly more frequent in women using ethinylestradiol-containing medicinal products such as CHCs. Additionally, also in patients treated with glecaprevir/pibrentasvir or sofosbuvir/velpatasvir/ voxilaprevir, ALT elevations were observed in women using ethinylestradiol- containing medicinal products such as CHCs. Women using medicinal products containing oestrogens other than ethinylestradiol, such as estradiol, and ombitasvir/paritaprevir/ritonavir and dasabuvir with or without ribavirin had a rate of ALT elevation similar to those not receiving any oestrogens; however, due to the limited number of women taking these other oestrogens, caution is warranted for co-administration with the following combination drug regimens: ombitasvir/paritaprevir/ritonavir and dasabuvir with or without ribavirin, glecaprevir/pibrentasvir or sofosbuvir/velpatasvir/voxilaprevir (see Section 4.5).

### **4.5 Interaction with Other Medicinal Products and Other Forms of Interaction**

No interaction studies have been performed.

#### **The efficacy of oestrogens and progestogens might be impaired:**

- The metabolism of oestrogens and progestogens may be increased by concomitant use of substances known to induce drug-metabolising enzymes, specifically the P450 enzymes 2B6, 3A4, 3A5, 3A7, such as anticonvulsants (e.g., phenobarbital, carbamazepine, phenytoin) and anti-infectives (e.g., rifampicin, rifabutin, nevirapine, efavirenz).
- Ritonavir and nelfinavir, although known as strong inhibitors of CYP450 3A4, A5, A7, by contrast, exhibit inducing properties when used concomitantly with steroid hormones.
- Herbal preparations containing St. John's Wort (*Hypericum perforatum*) may induce the metabolism of oestrogens and progestogens via the CYP450 3A4 pathway.

Clinically, an increased metabolism of oestrogens and progestogens may lead to decreased effect and changes in the uterine bleeding profile.

#### **Effect of HRT with oestrogens on other medicinal products**

Hormone contraceptives containing oestrogens have been shown to significantly decrease plasma concentrations of lamotrigine when co-administered due to induction of lamotrigine glucuronidation. This may reduce seizure control. Although the potential interaction between hormone replacement therapy and lamotrigine has not been studied, it is expected that a similar interaction exists, which may lead to a reduction in seizure control among women taking both medicinal products together.

### **Pharmacodynamic Interactions**

During clinical trials with the HCV combination drug regime ombitasvir/paritaprevir/ritonavir and dasabuvir with or without ribavirin, ALT elevations greater than 5 times the upper limit of normal (ULN) were significantly more frequent in women using ethinylestradiol-containing medicinal products such as CHCs. Additionally, also with glecaprevir/pibrentasvir or sofosbuvir/velpatasvir/ voxilaprevir, ALT elevations were observed in women using ethinylestradiol-containing medications such as CHCs. Women using medicinal products containing oestrogens other than ethinylestradiol, such as estradiol, and ombitasvir/paritaprevir/ritonavir and dasabuvir with or without ribavirin had a rate of ALT elevation similar to those not receiving any oestrogens; however, due to the limited number of women taking these other oestrogens, caution is warranted for co-administration with the following combination drug regimens: ombitasvir/paritaprevir/ritonavir and dasabuvir with or without ribavirin, glecaprevir/pibrentasvir or sofosbuvir/velpatasvir/voxilaprevir (see Section 4.4).

#### **Oestrogens might interfere with the metabolism of other drugs:**

Oestrogens per se may inhibit CYP450 drug-metabolising enzymes via competitive inhibition. This is in particular to be considered for substrates with a narrow therapeutic index, such as

- tacrolimus and cyclosporine A (CYP450 3A4, 3A3)
- fentanyl (CYP450 3A4)
- theophylline (CYP450 1A2).

Clinically this may lead to an increased plasma level of the affected substances up to toxic concentrations. Thus, careful drug monitoring for an extended period of time might be indicated and a dosage decrease of tacrolimus, fentanyl, cyclosporin A and theophylline may be necessary.

#### 4.6 Fertility, Pregnancy and Lactation

Femoston® Conti 0.5mg/2.5mg is not indicated during pregnancy. If pregnancy occurs during medication with Femoston® Conti 0.5mg/2.5mg treatment should be withdrawn immediately.

The results of most epidemiological studies to date relevant to inadvertent foetalexposure to combinations of oestrogens and progestogens indicate no teratogenic or foetotoxic effect. There are no adequate data from the use of estradiol/dydrogesterone in pregnant women.

#### Lactation

Femoston® Conti 0.5mg/2.5mg is not indicated during lactation.

#### 4.7 Effects on Ability to Drive and Use Machines

Femoston® Conti 0.5mg/2.5mg has no or negligible influence on the ability to drive and/or to use machines.

#### 4.8 Undesirable Effects

The most commonly reported adverse drug reactions of patients treated with estradiol/dydrogesterone in clinical trials are headache, abdominal pain, breast pain/ tenderness and back pain.

The following undesirable effects have been observed with the frequencies indicated below during clinical trials (n=5108):

| MedDRA system organ class                   | Very common (≥ 1/10) | Common (≥1/100 to <1/10) | Uncommon (≥1/1,000 to <1/100) | Rare (≥1/10,000 to <1/1,000) |
|---------------------------------------------|----------------------|--------------------------|-------------------------------|------------------------------|
| Infections and infestations                 |                      | Vaginal candidiasis      |                               |                              |
| Neoplasms benign, malignant and unspecified |                      |                          | Increase in size of leiomyoma |                              |
| Immune system disorders                     |                      |                          | Hypersensitivity              |                              |
| Psychiatric disorders                       |                      | Depression, nervousness  | Influence on libido           |                              |
| Nervous system disorders                    | Headache             | Migraine, dizziness      |                               |                              |
| Cardiac disorders                           |                      |                          |                               | Myocardial infarction        |

|                                                 |                         |                                                                                                                                                                                |                                                                                                                      |                              |
|-------------------------------------------------|-------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------|------------------------------|
| Vascular disorders                              |                         |                                                                                                                                                                                | Venous thromboembolism*                                                                                              |                              |
| Gastrointestinal disorders                      | Abdominal pain          | Nausea, vomiting, flatulence                                                                                                                                                   |                                                                                                                      |                              |
| Hepatobiliary disorders                         |                         |                                                                                                                                                                                | Abnormal hepatic function, occasionally with jaundice asthenia or malaise, and abdominal pain, gallbladder disorders |                              |
| Skin and subcutaneous tissue disorders          |                         | Allergic skin reactions (e.g. rash, urticaria, pruritus)                                                                                                                       |                                                                                                                      | Angioedema, vascular purpura |
| Musculoskeletal and connective tissue disorders | Back pain               |                                                                                                                                                                                |                                                                                                                      |                              |
| Reproductive system and breast disorders        | Breast pain/tenderness, | Menstrual disorders (including postmenopausal spotting, metrorrhagia, menorrhagia, oligo-/amenorrhoea, irregular menstruation, dysmenorrhoea), pelvic pain, cervical discharge | Breast enlargement, premenstrual syndrome                                                                            |                              |



|                                                     |  |                                                                     |                  |  |
|-----------------------------------------------------|--|---------------------------------------------------------------------|------------------|--|
| General disorders and administration site reactions |  | Asthenic conditions (asthenia, fatigue, malaise), peripheral oedema |                  |  |
| Investigations                                      |  | Increased weight                                                    | Decreased weight |  |

\*see below for further information

#### Breast cancer risk

- An up to 2-fold increased risk of having breast cancer diagnosed is reported in women taking combined oestrogen-progestogen therapy for more than 5 years.
- The increased risk in users of oestrogen-only therapy is lower than that seen in users of oestrogen-progestogen combinations.
- The level of risk is dependent on the duration of use (see Section 4.4).
- Absolute risk estimations based on results of the largest randomised placebo-controlled trial (WHI-study) and the largest meta-analysis of prospective epidemiological studies are presented.

#### **Largest meta-analysis of prospective epidemiological studies**

##### **Estimated additional risk of breast cancer after 5 years' use in women with BMI 27(kg/m<sup>2</sup>)**

| Age at start HRT (years)                                                                                                                                       | Incidence per 1000 never-users of HRT over a 5 year period (50-54 years) <sup>1</sup> | Risk ratio | Additional cases per 1000 HRT users after 5 years |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------|------------|---------------------------------------------------|
| <b>Oestrogen only HRT</b>                                                                                                                                      |                                                                                       |            |                                                   |
| 50                                                                                                                                                             | 13.3                                                                                  | 1.2        | 2.7                                               |
| <b>Combined oestrogen-progestogen</b>                                                                                                                          |                                                                                       |            |                                                   |
| 50                                                                                                                                                             | 13.3                                                                                  | 1.6        | 8.0                                               |
| Note: Since the background incidence of breast cancer differs by EU country, the number of additional cases of breast cancer will also change proportionately. |                                                                                       |            |                                                   |

<sup>1</sup> Taken from baseline incidence rates in England in 2015 women with BMI 27 (kg/m<sup>2</sup>)

### Estimated additional risk of breast cancer after 10 years' use in women with BMI 27(kg/m<sup>2</sup>)

| Age at start HRT (years) | Incidence per 1000 never users of HRT over a 10year period (50-59 years)* | Risk ratio                            | Additional cases per 1000 HRT users after 10 years |
|--------------------------|---------------------------------------------------------------------------|---------------------------------------|----------------------------------------------------|
|                          |                                                                           | <b>Oestrogen only HRT</b>             |                                                    |
| 50                       | 26.6                                                                      | 1.3                                   | 7.1                                                |
|                          |                                                                           | <b>Combined oestrogen-progestogen</b> |                                                    |
| 50                       | 26.6                                                                      | 1.8                                   | 20.8                                               |

\* Taken from baseline incidence rates in England in 2015 in women with BMI 27(kg/m<sup>2</sup>).

*Note:* Since the background incidence of breast cancer differs by EU country, the number of additional cases of breast cancer will also change proportionately.

| <b>US WHI studies - additional risk of breast cancer after 5 years' use</b>                                                                                                                                                     |                                                      |                                                        |                                                          |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------|--------------------------------------------------------|----------------------------------------------------------|
| Age range (yrs)                                                                                                                                                                                                                 | Incidence per 1000 women in placebo arm over 5 years | Risk ratio & 95%CI                                     | Additional cases per 1000 HRT users over 5 years (95%CI) |
|                                                                                                                                                                                                                                 |                                                      | <b>CEE oestrogen-only</b>                              |                                                          |
| 50 - 79                                                                                                                                                                                                                         | 21                                                   | 0.8 (0.7 – 1.0)                                        | -4 (-6 – 0) <sup>2</sup>                                 |
|                                                                                                                                                                                                                                 |                                                      | <b>CEE+MPA oestrogen &amp; progestogen<sup>‡</sup></b> |                                                          |
| 50 - 79                                                                                                                                                                                                                         | 17                                                   | 1.2 (1.0 – 1.5)                                        | +4 (0 – 9)                                               |
| <sup>‡</sup> When the analysis was restricted to women who had not used HRT prior to the study there was no increased risk apparent during the first 5 years of treatment: after 5 years the risk was higher than in non-users. |                                                      |                                                        |                                                          |

<sup>2</sup> WHI study in women with no uterus, which did not show an increase in risk of breast cancer

### **Endometrial cancer risk**

#### **Postmenopausal women with a uterus**

The endometrial cancer risk is about 5 in every 1000 women with a uterus not using HRT.

In women with a uterus, use of oestrogen-only HRT is not recommended because it increases the risk of endometrial cancer (see Section 4.4).

Depending on the duration of oestrogen-only use and oestrogen dose, the increase in risk of endometrial cancer in epidemiology studies varied from between 5 and 55 extra cases diagnosed in every 1000 women between the ages of 50 and 65.

Adding a progestogen to oestrogen-only therapy for at least 12 days per cycle can prevent this increased risk. In the Million Women Study the use of five years of combined (sequential or continuous) HRT did not increase the risk of endometrial cancer (RR of 1.0 (0.8-1.2)).

### **Ovarian cancer**

Use of oestrogen-only or combined oestrogen-progestogen HRT has been associated with a slightly increased risk of having ovarian cancer diagnosed (see Section 4.4).

A meta-analysis from 52 epidemiological studies reported an increased risk of ovarian cancer in women currently using HRT compared to women who have never used HRT (RR 1.43, 95% CI 1.31-1.56). For women aged 50 to 54 years taking 5 years of HRT, this results in about 1 extra case per 2000 users. In women aged 50 to 54 who are not taking HRT, about 2 women in 2000 will be diagnosed with ovarian cancer over a 5-year period.

### **Risk of venous thromboembolism**

HRT is associated with a 1.3- to 3-fold increased relative risk of developing venous thromboembolism (VTE), i.e., deep vein thrombosis or pulmonary embolism. The occurrence of such an event is more likely in the first year of using HRT (see Section 4.4). Results of the WHI studies are presented:

#### **WHI Studies - Additional risk of VTE over 5 years' use**

| Age range (years)                    | Incidence per 1000 women in placebo arm over 5 years | Risk ratio and 95%CI | Additional cases per 1000 HRT users |
|--------------------------------------|------------------------------------------------------|----------------------|-------------------------------------|
| Oral oestrogen-only <sup>3</sup>     |                                                      |                      |                                     |
| 50 - 59                              | 7                                                    | 1.2 (0.6-2.4)        | 1 (-3 – 10)                         |
| Oral combined oestrogen- progestogen |                                                      |                      |                                     |
| 50 - 59                              | 4                                                    | 2.3 (1.2 – 4.3)      | 5 (1 - 13)                          |

<sup>3</sup> Study in women with no uterus

### **Risk of coronary artery disease**

The risk of coronary artery disease is slightly increased in users of combined oestrogen- progestogen HRT over the age of 60 (see Section 4.4).

### **Risk of ischaemic stroke**

The use of oestrogen-only and oestrogen+progestogen therapy is associated with an up to 1.5-fold increased relative risk of ischaemic stroke. The risk of haemorrhagic stroke is not increased during use of HRT.

This relative risk is not dependent on age or duration of use, but as the baseline risk is strongly age-dependent, the overall risk of stroke in women who use HRT will increase with age (see Section 4.4).

#### **WHI studies combined - Additional risk of ischaemic stroke<sup>4</sup> over 5 years' use**

| Age range (years) | Incidence per 1000 women in placebo arm over 5 years | Risk ratio and 95%CI | Additional cases per 1000 HRT users over 5 years |
|-------------------|------------------------------------------------------|----------------------|--------------------------------------------------|
| 50 - 59           | 8                                                    | 1.3 (1.1 - 1.6)      | 3 (1 - 5)                                        |

<sup>4</sup> No differentiation was made between ischaemic and haemorrhagic stroke

Other adverse reactions have been reported in association with oestrogen/progestogen treatment (including estradiol/dydrogesterone):

#### **Neoplasms benign, malignant and unspecified:**

Oestrogen dependent neoplasms both benign and malignant, e.g., endometrial cancer, ovarian cancer. Increase in size of progestogen dependent neoplasms, e.g., meningioma.

#### **Blood and lymphatic system disorders:**

Haemolytic anaemia

Immune system disorders:

Systemic lupus erythematosus

Metabolism and nutrition disorders:

Hypertriglyceridemia

Nervous system disorders:

Probable dementia, chorea, exacerbation of epilepsy

Eye disorders:

Steepening of corneal curvature, contact lenses intolerance

Vascular disorders:

Arterial thromboembolism

Gastrointestinal disorders:

Pancreatitis (in women with pre-existing hypertriglyceridemia)

Skin and subcutaneous tissue disorders:

Erythema multiforme, erythema nodosum, chloasma or melasma, which may persist when drug is discontinued.

Musculoskeletal and connective tissue disorders:

Leg cramps

Renal and urinary disorders:

Urinary incontinence

Reproductive system and breast disorders:

Fibrocystic breast disease, uterine cervical erosion

Congenital, familial and genetic disorders:

Aggravated porphyria

Investigations:

Total thyroid hormones increased

#### **4.9 Overdose**

Both estradiol and dydrogesterone are substances with low toxicity. Symptoms such as nausea, vomiting, breast tenderness, dizziness, abdominal pain, drowsiness/fatigue, and withdrawal bleeding could occur in cases of overdosing. It is unlikely that any specific symptomatic treatment will be necessary. Aforementioned information is also applicable for overdosing in children.

## **5. PHARMACOLOGICAL PROPERTIES**

### **5.1 Pharmacodynamic properties**

Pharmacotherapeutic group: Genito urinary system and sex hormones, progestogens and oestrogens, fixed combinations. The ATC code is G03FA14.

### **Estradiol**

The active ingredient, 17 $\beta$ -estradiol, is chemically and biologically identical to endogenous human estradiol.

It substitutes for the loss of oestrogen production in menopausal women and alleviates menopausal symptoms.

### **Dydrogesterone**

Dydrogesterone is an orally-active progestogen having an activity comparable to parenterally administered progesterone.

As oestrogens promote the growth of the endometrium, unopposed oestrogens increase the risk of endometrial hyperplasia and cancer. The addition of a progestogen greatly reduces the oestrogen-induced risk of endometrial hyperplasia in non-hysterectomised women.

### **Clinical trial information**

- Relief of oestrogen-deficiency symptoms and bleeding patterns
- Relief of menopausal symptoms was achieved during the first few weeks of treatment

With Femoston® conti 0.5 mg/2.5 mg the reduction of moderate to severe hot flushes was statistically significant versus placebo from week 4 onward. The number of moderate to severe hot flushes decreased further until end of treatment period in week 13.

In two studies amenorrhoea (no bleeding or spotting) was seen in 91% and in 88% of the women respectively during months 10-12 of treatment. Irregular bleeding and or spotting appeared in 10% and 21% of the women during the first 3 months of treatment and in 9% and in 12% during months 10-12 of treatment.

## **5.2 Pharmacokinetic properties**

### **Estradiol**

- Absorption:

Absorption of estradiol is dependent on the particle size: micronized estradiol is readily absorbed from the gastrointestinal tract.

The following table provides the mean single dose pharmacokinetic parameters of estradiol (E2), estrone (E1) and estrone sulphate (E1S) for each dose of micronized estradiol. Data is presented as mean (SD).

| <b>Estradiol 0.5mg</b>       |             |             |                              |             |
|------------------------------|-------------|-------------|------------------------------|-------------|
| Parameters                   | E2          | E1          | Parameters                   | E1S         |
| C <sub>max</sub> (pg/ml)     | 34.8 (30.4) | 182 (110)   | C <sub>max</sub> (ng/ml)     | 6.98 (3.32) |
| C <sub>min</sub> (pg/ml)     | -           | -           | -                            | -           |
| C <sub>av</sub> (pg/ml)      | 21.5 (16.0) | -           | -                            | -           |
| AUC <sub>0-t</sub> (pg.h/ml) | 516 (383)   | 2959 (2135) | AUC <sub>0-t</sub> (ng.h/ml) | 82.0 (42.6) |

- Distribution:

Oestrogens can be found either unbound or bound. About 98- 99% of the estradiol dose binds to plasma proteins, from which about 30-52% on albumin and about 46-69% on the sex hormone-binding globulin (SHBG).

- Metabolism:

Following oral administration, estradiol is extensively metabolised. The major unconjugated and conjugated metabolites are estrone and estrone sulphate. These metabolites can contribute to the oestrogen activity, either directly or after conversion to estradiol. Estrone sulphate may undergo enterohepatic circulation.

- Elimination:

In urine, the major compounds are the glucuronides of estrone and estradiol. The elimination half-life is between 10-16 h.

Oestrogens are secreted in the milk of nursing mothers.

- Dose and time dependencies:

Following daily oral administration of Femoston® Conti 0.5mg/2.5mg, estradiol concentrations reached a steady-state after about five days.

Generally, steady state concentrations appeared to be reached for within 8 to 11 days of dosing.

## Dydrogesterone

- Absorption:

Following oral administration, dydrogesterone is rapidly absorbed with a  $T_{max}$  between 0.5 and 1.5 hours.

The following table provides the mean steady state pharmacokinetic parameters of dydrogesterone (D) and dihydrodydrogesterone (DHD). Data is presented as mean (SD).

| <b>Dydrogesterone 2.5 mg</b> |                 |             |
|------------------------------|-----------------|-------------|
| <i>Parameters</i>            | <i>D</i>        | <i>DHD</i>  |
| $C_{max}$ (ng/ml)            | 0.759 (0.313)   | 18.9 (7.22) |
| $C_{min}$ (ng/ml)            | 0.0309 (0.0209) | -           |
| $C_{av}$ (ng/ml)             | 0.117 (0.0455)  | -           |
| AUC 0-t (ng.h/ml)            | 2.81 (1.09)     | 90.4 (44.1) |

After a single dose, food delays the peak plasma concentration of dydrogesterone with approximately 1 hour, resulting in approximately 20% lower dydrogesterone peak plasma concentrations without affecting the extent of exposure to dydrogesterone and DHD.

- Distribution:

After oral administration of dydrogesterone the apparent volume of distribution is large, being approximately 22000 L. Dydrogesterone and DHD are more than 90% bound to plasma proteins.

- Metabolism:

Following oral administration, dydrogesterone is rapidly metabolised to DHD. The levels of the main active metabolite 20  $\alpha$ -dihydrodydrogesterone (DHD) peak at similar times as dydrogesterone. The plasma levels of DHD are substantially higher as compared to the parent drug. The AUC and  $C_{max}$  ratios of DHD to dydrogesterone are approximately 25 and 20 respectively. The mean terminal half-lives of dydrogesterone and DHD is about 15 hours. A common feature of all metabolites characterised is the retention of the 4,6 diene-3-one configuration of the parent compound and the absence of 17 $\alpha$ -hydroxylation. This explains the lack of oestrogenic and androgenic effects of dydrogesterone.

- Elimination:

After oral administration of labelled dydrogesterone, on average 63% of the dose is excreted into the urine. The apparent total body clearance of dydrogesterone from plasma is high at approximately 20 L/min. Within 72 hours excretion is complete. DHD is present in the urine predominantly as the glucuronic acid conjugate.

- Dose and time dependencies:

The single and multiple dose pharmacokinetics are linear in the oral dose range 2.5 to 20 mg. Comparison of the single and multiple dose kinetics shows that the pharmacokinetics of dydrogesterone and DHD are not changed as a result of repeated dosing. Steady state conditions are generally reached after 3 days of treatment.

### 5.3 Preclinical Safety Data

There are no preclinical safety data of relevance in the target population that are additional to those already included in other sections.

## 6. PHARMACEUTICAL PARTICULARS

### 6.1 List of excipients

#### Core:

Lactose monohydrate

Hypromellose

Maize starch

Colloidal anhydrous silica

Magnesium stearate

#### Film-coating:

| Formulation                                   | Tablet colour | Composition of Colouring agents                                                                   |
|-----------------------------------------------|---------------|---------------------------------------------------------------------------------------------------|
| 0.5 mg estradiol and<br>2.5 mg dydrogesterone | yellow        | Titanium dioxide (E171)<br>Iron oxide yellow (E172)<br>Polyvinyl alcohol<br>Macrogol 3350<br>Talc |

### 6.2 Incompatibilities

Not applicable.

### 6.3 Shelf Life

3 years.

### 6.4 Storage Condition

Store below 30°C.

### 6.5 Nature and Contents of Container

PVC/Aluminium blister strips in a printed carton.

Blister packs: 28 film-coated tablets  
84 film-coated tablets  
280 (10 x 28) film-coated tablets

Not all pack sizes may be marketed.

### 6.6 Special Precautions for Disposal and Other Handling

Any unused product or waste material should be disposed of in accordance with local requirements.

**7. MANUFACTURER:**

Abbott Biologicals B.V. Veerweg  
12, NL-8121AA Olst, The Netherlands

**8. DATE OF REVISION OF THE TEXT**

September 2025

RDCCDS000421/16